

Master 06 — Phase 0/1

Operating Model

Audience: Programme operations, commissioners, governance leads, delivery partners, technical supplier, evaluation partner.

Purpose: Define how the first intervention test runs from readiness through Week-12 evidence.

Status: DRAFT — needs Dean review.

Priority: P1.

Review needed from: Dean; governance; IG / DPO; safeguarding; clinical route; operations; evaluation.

Source basis: Master 06 source text, Programme Plan v0.1, Programme Lock, START update.

A4 REVIEW PAGE 1 — Operating model opening

This document describes the operating model for the first OTOS Continuity™ intervention test.

This is not a generic continuity pilot. It is the first bounded test of the **OTOS Continuity Management Layer** and user **Continuity Thread** in a suspected ADHD-driven addiction cohort:

1. identify adults whose addiction, relapse or repeated crisis may be driven by untreated or inadequately treated ADHD;
2. route the ADHD driver toward qualified clinical assessment and treatment where clinically indicated;
3. provide the OTOS Continuity Management Layer around the person so authorised partners can manage their agreed part of continuity, while the user's Continuity Thread follows the whole-person journey over time through ADHD clinical-route status, addiction support, structured support / waiting-well activity, handoffs, behavioural reinforcement and habit-stacking practice;
4. evidence what happened;

5. decide whether to continue, continue with changes, rebuild, pause or stop.

The first proposed test is a governed Cambridge / Cambridgeshire and Peterborough intervention demonstrator for up to 50 adults, with a 12-week active support window and follow-up evidence at 3 and 6 months where consent, governance and partner data-sharing allow.

It does not require Electronic Patient Record (EPR) integration. It does not automate clinical decisions. It does not ask partners to endorse a finished national platform.

It asks whether a suspected ADHD-driven addiction cohort can be found, clinically routed, supported through authorised partner activity in the Continuity Management Layer, followed through a whole-person Continuity Thread, and evidenced safely.

START strengthens the evidence rationale because it points to a real-world dual-diagnosis service-journey signal. Its lesson is not that medication alone fixes addiction. Its lesson is that ADHD identification, medically supervised treatment where appropriate, addiction support, engagement, monitoring and follow-up need to be held together as one person's journey with multiple service requirements.

OTOS provides the non-clinical infrastructure for that test in plain service terms:

- a consent-led Continuity Management Layer around the person;
- authorised partner access into that Layer;
- a Continuity Thread showing the user's whole-person journey over time through agreed non-clinical events;
- touchpoint logs showing whether contact happened;
- handoff receipts showing whether the next step landed;
- partner-approved SMS check-ins and prompts;
- delivery, failure, reply, silence and opt-out evidence;
- drift / silence signals for human review;
- role-based dashboards for the agreed team;
- evidence exports and audit logs.

It should be recommended that addiction-service leads and the ADHD clinical route meet at the start of Phase 0 to agree what can be shared, what cannot be shared, and which minimum status categories are safe. If the ADHD team cannot participate directly at the first stage, the model can still run with the addiction-service partner only, but the operating model must then be adjusted so OTOS records the participant's consent-led status and does not imply clinical ADHD-team participation.

A4 REVIEW PAGE 2 — Twelve-step operating sequence

This table explains the practical flow in plain language. It should be readable by service providers who do not already know OTOS.

Step	What happens in practice	Evidence created	Boundary
1	A partner worker recognises a possible ADHD-driven addiction pattern in an adult already known to, or entering, the service.	Source touchpoint, date, partner role and reason for possible fit.	This is recognition of a suspected pattern only. OTOS does not diagnose ADHD or addiction.
2	The person is given a clear explanation of OTOS Continuity™: what it does, what it does not do, who may access agreed information and which authorised Thread elements may be visible and how they can opt out.	Information-provided timestamp and version of participant explanation used.	Consent-led support only. No pressure to join and no effect on normal care if they decline.
3	The person gives consent for the Continuity Management Layer, SMS contact and agreed Thread visibility.	Consent record, SMS permission, opt-out route and withdrawal terms.	Withdrawal and opt-out must be clear. Consent does not authorise unnecessary clinical sharing.
4	A participant record is created in the Continuity	Participant token / thread record and	This is not a shared clinical record and

Step	What happens in practice	Evidence created	Boundary
	Management Layer. This opens the user's Continuity Thread so agreed partners can view or add authorised service-journey status, touchpoints and continuity events.	access settings.	does not replace any NHS or partner record. Partners see only authorised Thread elements relevant to their role.
5	A baseline continuity snapshot is recorded: current addiction-service position, ADHD clinical-route status, support route, next known step and agreed continuity risks / service-journey gaps using partner-approved categories only.	Baseline snapshot with status fields and next-step date where known.	Continuity information only. No diagnosis, therapy notes or prescribing decisions.
6	The ADHD clinical route is recorded in status terms: for example, not yet referred, referral discussed, referral made, appointment booked, appointment attended, outcome / next step known where consent allows.	ADHD clinical-route status and clinical-owner field.	Qualified clinical teams own assessment, diagnosis, prescribing and monitoring. OTOS only records agreed status.
7	The addiction-service route is recorded: keyworker contact, recovery-plan touchpoints, groups, community recovery / CRS touchpoints and agreed next actions.	Addiction-service touchpoints, attendance / non-attendance and next-step receipts.	Addiction service retains responsibility. OTOS does not provide addiction treatment.
8	Structured support and waiting-well routes are added where agreed:	Engagement, attendance, completion or	No private therapy notes. Partner-approved prompts

Step	What happens in practice	Evidence created	Boundary
	CBT / DCBT, psychoeducation, recovery learning, breathwork, meditation, short activity prompts, community touchpoints or partner content.	non-engagement signals.	may be delivered through the Continuity Management Layer. OTOS provides the infrastructure and audit trail; it does not provide therapy or clinical support.
9	Handoffs are logged whenever the person is meant to move from one contact or service step to another.	Landed / not landed / unknown status, date, responsible partner and next action.	A receipt is not a clinical judgement. It only shows whether the intended contact or step appears to have happened.
10	Partner-approved mobile SMS check-ins and prompts are delivered through the Continuity Management Layer according to the agreed schedule and wording.	Sent, delivered, failed, replied, silence, help requested and opt-out data.	Messages are non-clinical prompts / receipts. They must not contain diagnosis, prescribing advice or crisis-service substitution.
11	A human reviews drift, silence, missed handoffs, help requests or agreed concern signals during the operating rhythm.	Human-review record, review outcome category and timestamped action / escalation status where agreed.	No automated clinical triage. Safeguarding and clinical escalation follow partner-approved protocols.
12	At Week 12, the active-support evidence is exported and reviewed. Follow-up evidence is then planned for 3 and 6 months where consent, governance and partner data-sharing allow.	Week-12 evidence pack, decision paper, 3-month / 6-month follow-up plan.	Decision-grade local evidence, not national proof. The programme decides continue / continue with changes / rebuild / pause / stop.

Dean review note: This page should stay detailed enough that partners can understand what a human does, what the participant experiences, what evidence is created and where OTOS stops. Avoid unexplained internal terms. Use “service journey” and “clinical route” rather than implying OTOS owns a clinical pathway.

A4 REVIEW PAGE 3 — Phase 0 readiness

Purpose: prepare the intervention test so it is safe, bounded, low-burden and clear before any participant enters.

Phase 0 must produce:

- partner role confirmation sheet;
- cohort criteria;
- inclusion / exclusion checklist;
- clinical boundary statement;
- safeguarding protocol;
- crisis-language SMS wording;
- participant information sheet;
- consent form;
- SMS consent and opt-out wording;
- DPIA draft;
- DPA draft;
- controller / processor position for review;
- SMS template set;
- dashboard specification;
- evidence export template;
- partner training pack;
- weekly review agenda;
- risk register;
- Week-12 report template;

- continue / continue with changes / rebuild / pause / stop decision paper.

Phase 0 gateway: no build should proceed until the programme has agreed cohort criteria, partner roles, clinical boundaries, safeguarding thresholds, IG route, SMS rules, evaluation framework and funding route.

A4 REVIEW PAGE 4 — Phase 1 running model

Purpose: test the intervention sequence for up to 50 adults over 12 weeks.

Period	Activity	Key outputs
Week 0	Final readiness, Layer / service-line configuration, staff access, template approval	Launch readiness checklist
Weeks 1–2	Mobilisation, staff onboarding, SMS testing, review queue testing, dummy records	Tested workflows; known issues log
Weeks 3–4	Participant onboarding, consent, baseline snapshots, initial touchpoints	Cohort onboarding report
Weeks 5–10	Active running: SMS, handoffs, drift review, weekly operating rhythm	Weekly dashboard; issue log
Weeks 11–12	Close open items, collect partner feedback, export evidence	Week-12 evidence pack
End gate	Formal decision	Continue / continue with changes / rebuild / pause / stop recommendation

The operating rhythm should include:

- weekly dashboard review;
- open drift / silence review;
- handoff receipt review;
- SMS performance review;
- safety / incident check;
- partner-burden check;
- data-quality check;

- decisions log.
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A4 REVIEW PAGE 5 — SMS, data and human review

SMS is part of the intervention infrastructure because continuity cannot depend on people remembering to log into a portal.

SMS creates small, auditable signals:

- sent;
- delivered;
- failed;
- replied;
- opted out;
- no response;
- help requested;
- handoff acknowledged.

SMS creates events on the user's Continuity Thread through the Continuity Management Layer. It does not carry clinical judgement.

SMS must not include diagnosis, medication advice, prescribing guidance, sensitive clinical details, therapy content, shame-based language or emergency-service substitution.

Any structured support prompts delivered through the Continuity Management Layer must use partner-approved wording and must remain non-clinical.

The first build should support:

- participant tokenisation;
- consent capture;
- partner / source service-line field;
- ADHD clinical-route status field;
- medication-status category where agreed;
- addiction-service touchpoints;
- structured support / waiting-well touchpoints;

- handoff receipts;
- partner-approved SMS check-in schedules;
- inbound response handling;
- drift / silence rules;
- human-review queue;
- role-based dashboard;
- evidence export;
- audit log;
- role-based access.

Every agreed drift or concern signal must be routed to human review. The system must not automate clinical triage.

A4 REVIEW PAGE 6 — Week-12 evidence and stop logic

The Week-12 evidence pack should be the required build export. It should draw from agreed Layer activity and authorised Thread events. It must evidence continuity signals and service-journey activity, not clinical outcomes or full partner records. It should also sit inside a wider evidence framework that can support planned 3-month and 6-month follow-up where consent, governance and partner data-sharing allow.

The Week-12 evidence pack should include:

1. cohort summary;
2. consent and withdrawal summary;
3. ADHD clinical-route / next-step status summary where agreed;
4. medication-status category summary where agreed;
5. addiction-service touchpoint report;
6. structured support / waiting-well touchpoint report;
7. handoff receipt report;
8. SMS delivery / failure / reply / silence report;
9. drift / silence report;

10. human-review report;
11. partner burden report;
12. safety / incident report;
13. qualitative partner feedback;
14. evidence-labelled claims table;
15. continue / continue with changes / rebuild / pause / stop recommendation.

The programme must be capable of stopping.

Continue if the evidence is credible and the burden / risk is tolerable.

Continue with changes if the model is promising but needs alteration.

Rebuild if the technical, governance or workflow model is not yet good enough.

Pause if governance, capacity, safety or funding is unclear.

Stop if the cohort cannot be identified safely, the boundary cannot be protected, or the evidence does not justify further spend.

Review notes

Dean decision needed

- ☒ ~~Confirm whether the document title should be "Phase 0/1 Operating Model" or "First Intervention Test Operating Model".~~

Dean note: Phase 0/1 Operating Model - Intervention Test.

- ☒ ~~Confirm whether named local partners appear in this operating model or only in partner-specific briefs.~~

Dean note: Name the organisations not the individuals.

- ☒ ~~Confirm whether the Week 12 evidence pack list is approved.~~

Dean note: Not approved yet. The evidence pack needs to be reconsidered as a wider evidence framework, not just a Week-12 pack. It must include the 12-week active-support evidence and the planned 3-month and 6-month follow-up elements where consent, governance and partner data-sharing allow.

Evidence / citation check needed

- ☒ ~~START citation if retained in the opening.~~

Dean note: Tick in principle. START can stay in the opening because it supports the rationale for testing the full service journey: ADHD identification, clinical routing, treatment where appropriate, addiction support, engagement, monitoring and follow-up. Final external copy must insert the exact START citation and use the Master 09 caveats. Do not present START as medication-only proof, an OTOS outcome claim, or a prediction that OTOS will replicate 61.3%.

- ☒ ~~Evidence-check any mechanism claims before external use.~~

Dean note: Tick in principle. The mechanism claims are directionally right, but they must be evidence-checked before external use. Keep the wording guarded: suspected ADHD-driven addiction, where clinically indicated, clinical teams own assessment / treatment, OTOS is non-clinical continuity infrastructure, and local evidence must be generated. Check any claims about medication, CBT, recovery learning, relapse, engagement, START, or improved outcomes against Master 09 before publication.

Clinical / governance check needed

- ☐ Full governance review required before use.
- ☐ Confirm clinical-owner field wording.
- ☐ Confirm medication-status field wording.
- ☐ Confirm safeguarding and out-of-hours process.
- ☐ Confirm controller / processor position.

Dean note: Leave these unticked for now. These are not Dean-only decisions. They require input from the relevant clinical, safeguarding, IG / DPO and governance reviewers before approval. Dean has reviewed the section, but final sign-off should wait for the clinical / governance team.

Copy approval

- ☐ Approved
 - ☒ ~~Needs edit~~
 - ☐ Hold
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